

PROJECT PROPOSAL

Shesher Kobita Ananda Ashram

A Dignified Residential Care and Service Centre for Older Persons



Figure 1. Conceptual architectural rendering supplied with the project proposal.

Submitted by

MITALI HOSSAIN TRUST

Funding request: BDT 27,982,500 for one-time construction and establishment costs

Funding at a Glance

BDT 27.98m	60	11,000 sq ft	22 decimals
one-time capital request	initial resident capacity	two-storey built area	Trust-owned donated land

Purpose of this request. Mitali Hossain Trust seeks funding solely for the one-time construction and establishment of a two-storey residential care centre in Panchgaon Mouza, Arai hazar Upazila, Narayanganj District. Following completion, affordable monthly resident fees are intended to cover routine operations, including staffing, food, utilities, and maintenance.

Introduction: The Situation of Older Persons in Bangladesh

Bangladesh's demographic structure is changing rapidly. A country once characterised by a predominantly young population is experiencing a growing proportion of older persons. According to the World Bank data cited in the source proposal, by 2025 one in every ten citizens of Bangladesh would be aged 60 years or older, and this proportion is expected to rise more rapidly in the coming decades. The proportion of economically dependent older persons also increased from 8 per cent in 2019 to 9.4 per cent in 2023. In practical terms, the number of older people who are no longer economically active and who depend on others is steadily increasing. This demographic transition has occurred alongside major disruption in social and family structures.

The shift from extended to nuclear families, the movement of adult children within Bangladesh and abroad for employment, urbanisation, and increasingly demanding lifestyles have left many older persons separated from family support in later life. Public statements made around the International Day of Older Persons have acknowledged this challenge: loneliness and social isolation, together with age-related physical and mental health concerns, can intensify in later life. Government, private initiatives, and individuals with financial means all have a role in enabling older persons to live with dignity.



Figure 2. Illustrative image: family separation in later life.



Figure 3. Illustrative image: social isolation among older persons.

This challenge is particularly acute among older people from middle- and lower-middle-income households. Wealthier families may purchase costly private care, while the poorest older persons may qualify for government old-age allowances and social welfare programmes. Between these groups is a substantial population with modest savings or income but no dependable caregiver in later life. Government eligibility guidance for the old-age allowance also recognises this vulnerability by prioritising, in sequence, widows, divorced women, widowers, childless individuals, and people separated from their families.

In practice, many unmarried older persons, widows and widowers, childless couples, and parents whose children have settled elsewhere in Bangladesh or abroad may possess savings or pensions but still lack reliable assistance during illness, accidents, or emergencies. Institutional capacity is also limited. The source proposal states that Bangladesh has approximately 32 registered and private homes for older persons, of which only 6 are

government-owned. This is inadequate in relation to a large and growing older population. Most private facilities are designed for affluent residents and remain beyond the reach of middle- and lower-middle-income households. Consequently, a large population faces severe insecurity in later life despite having some financial means, because appropriate, affordable, and dignified accommodation is unavailable. Shesher Kobita Ananda Ashram is designed to address this service gap.

Source note: The demographic, policy, and sector figures above are reproduced from the Bengali source proposal. Although that proposal attributes demographic data to the World Bank, it does not supply full bibliographic references. Authoritative citations should be appended by the applicant as part of donor due diligence.

Executive Summary

In response to the circumstances described above, Mitali Hossain Trust proposes to establish **Shesher Kobita Ananda Ashram**, a dignified residential care and service centre for older persons, on 22 decimals of Trust-owned land (0.22 acres; approximately 890 square metres) in Panchgaon Mouza, Araihaaz Upazila, Narayanganj District. The centre will primarily serve older persons from middle- and lower-middle-income families, especially those without family support, including widowed, unmarried, and childless individuals and those who have no one to care for them in later life.

The first phase will construct a two-storey building comprising 5,500 square feet on each floor, for a total built area of 11,000 square feet. The initial facility will accommodate 60 older residents at one time, with scope for future expansion according to demand. Total estimated construction and establishment costs are **BDT 27,982,500** (2 crore 79 lakh 82 thousand 5 hundred taka). This proposal requests funding only for that one-time capital cost.

After establishment, affordable monthly fees paid by residents will finance the centre's day-to-day operations, supporting long-term financial self-reliance. The Trust therefore appeals to compassionate individuals, institutional donors, foundations, and responsible businesses for financial partnership in meeting the one-time construction requirement.



Figure 4. Conceptual front-elevation rendering of Shesher Kobita Ananda Ashram.

1. Trust Profile and Background

Mitali Hossain Trust is a non-profit charitable organisation dedicated to the welfare of vulnerable, low-income, and older people. Its founder and lifetime Chair, Mitali Hossain, first envisioned dignified accommodation for older persons approximately a decade ago. After a long period of waiting, she decided to begin the initiative by donating her own land. That decision led to the establishment of Mitali Hossain Trust.



Figure 5. Preliminary project meeting supplied with the source proposal.



Figure 6. Project event image supplied with the source proposal.

2. Core Objectives under the Trust Deed

- Establish and operate Shesher Kobita Ananda Ashram as a dignified residential care and service centre for older persons.
- Apply the Trust's income to the welfare of low-income and vulnerable women, men, and children, and to environmental conservation.
- Operate and preserve Smriti 71 Library, founded by Mitali Hossain. The library has already been separately established and is in operation; it is not part of the building proposed in this project.
- Bring any future service institution under the governance of the Trust.
- Undertake activities in education, health, poverty reduction, and environmental conservation.

The 22 decimals of land donated for the project, recorded under Deed No. 6016, have an estimated current market value of **BDT 14,000,000** (1 crore 40 lakh taka). The land has already been transferred into the Trust's name as its initial capital contribution.

3. Board of Trustees

Position	Name
Chair	Mitali Hossain
Vice-Chair	Moutusi Khandakar
Secretary-General	Advocate Rumana Yasmin Shaon
Joint Secretary	Khandakar Abul Ehsan
Treasurer	Kamrun Nahar
General Members	Joharul Mamun; Imelda Hossain; Dr Farhana Nashid Rakhi; Md Shah Ali Newaz Topu

4. Project Location and Land

The project is located in Panchgaon Mouza, Duptara Union, Araihaazar Upazila, Narayanganj District. The building will be constructed on 22 decimals of land owned by the Trust. Its relative proximity to metropolitan Dhaka will facilitate regular visits by family members and relatives, while its distance from urban congestion will provide a peaceful and healthy setting for older residents.



Figure 7. Project site and landscape plan supplied with the source proposal.

5. Building Plan, Room Configuration and Capacity

Project parameter	Specification
Ground floor	5,500 square feet
First floor	5,500 square feet
Total built area	11,000 square feet
Initial capacity	60 older residents at one time; expandable in response to future demand

The **ground floor** will contain reception and administrative offices, a dining hall and kitchen, a television room, a separate medical/sick room, and separate well-lit and well-ventilated rooms for older residents. The **first floor** will contain additional residential rooms, common sitting and social areas, and a spacious veranda/open area. Wide wheelchair-accessible corridors, handrails at appropriate heights, and age-friendly design principles will be applied throughout the building.

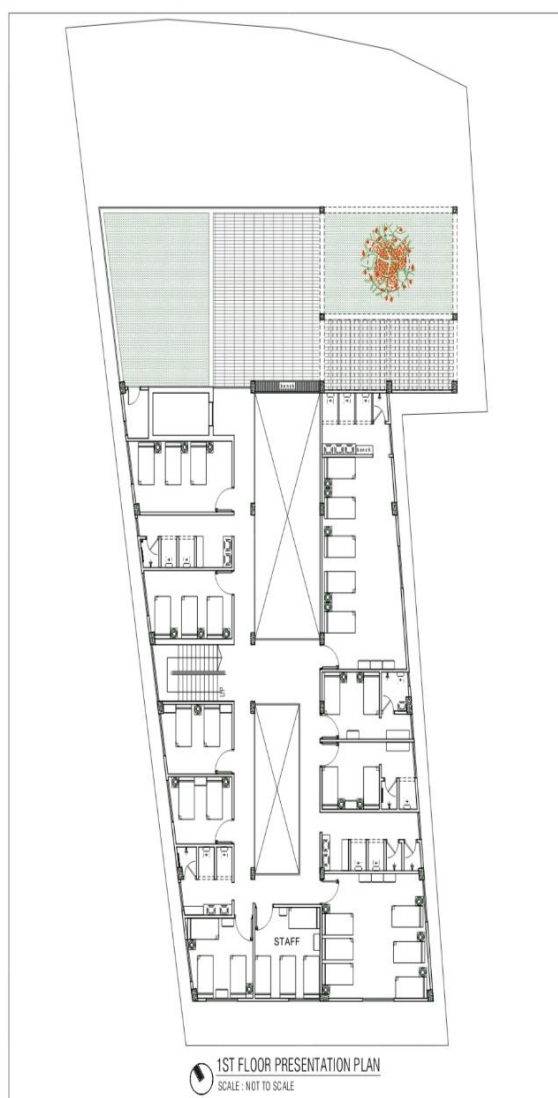


Figure 8. First-floor presentation plan (not to scale), supplied with the source proposal.

6. Facilities and Services

- Clean, well-lit, well-ventilated, furnished and habitable rooms.
- Twenty-four-hour trained care staff and supervision.
- Emergency medical support and arrangements for rapid transfer to hospital.
- A separate sick room with specialised care.
- Wheelchair-accessible, age-friendly design.
- Uninterrupted electricity supported by a solar power system.
- Closed-circuit camera security system.
- Regular opportunities for family visits and communication.
- Nutritious and easily digestible meals.
- Social and cultural activities, including conversation, religious discussion, indoor games, and festival observance.
- Open gardens and sitting areas shaded by surrounding neem trees.



Figure 9. Conceptual rendering of a shared residential room.



Figure 10. Conceptual rendering of a residential room and corridor.



Figure 11. Conceptual rendering of the landscaped courtyard.



Figure 12. Conceptual rendering of the dining and community room.

7. Proposed Staffing Structure

Role	Number
Chief Executive Officer	1
Superintendent-cum-Accountant	1
Caregivers	4
Nurses	2
Kitchen staff	3
Office attendant	1
Cleaning staff	2
Security guards	2
Gardener	1
Total	17

Staff salaries and all other day-to-day operating expenses will be met from residents' monthly fees. These recurring costs are not included in the present fundraising request.

8. Estimated Construction and Establishment Budget

The budget was prepared using prevailing 2026 market rates in Bangladesh. All amounts are in Bangladeshi taka (BDT).

Budget item / description	Quantity / basis	Estimated cost (BDT)
Main building - ground floor, including foundation, RCC structure, masonry, roof casting, plastering, finishing, doors and windows, electrical and water lines, and septic tank	5,500 sq ft x BDT 2,500	13,750,000
Main building - first floor (excluding foundation)	5,500 sq ft x BDT 1,800	9,900,000
Wheelchair railings and age-friendly structural adaptations	Estimated	300,000
Solar power system (panels, inverter and batteries)	Estimated	500,000
Generator backup system	Estimated	300,000
Closed-circuit cameras and security system	Estimated	100,000
Equipment for medical/sick room	Estimated	200,000
Kitchen and canteen equipment; television room	Estimated	300,000
Furniture - beds, wardrobes, chairs, tables and common-room furnishings	Estimated	1,000,000
Curtains and light interior decoration	Estimated	300,000
Contingency (approximately 5%)		1,332,500
TOTAL		27,982,500

Total estimated cost: BDT 27,982,500 - twenty-seven million nine hundred eighty-two thousand five hundred Bangladeshi taka (2 crore 79 lakh 82 thousand 5 hundred taka).

9. Sustainability of Operating Costs

After construction is complete, recurring expenses - including staff salaries, food, electricity, water, and maintenance - will be financed through affordable monthly fees paid by residents. Because the centre is intended for middle- and lower-middle-income households, fees will be set at a realistic and affordable level, substantially below those charged by commercial homes for older persons.

Residents who, after a long period of residence, lose the ability to pay will be subsidised from income generated by the Sadaqah Jariyah fund described in Section 10(b). Under this model, once the centre has been built, it is not intended to rely on repeated annual donations. Donor contributions will therefore create a permanent institution with a pathway to operational self-reliance.

10. Donation and Fundraising Mechanisms for Construction

Donors may participate in the one-time construction cost through any of the following mechanisms.

10(a). Lifetime Residential Rights

A donor may secure the lifelong right to occupy a room for an older person designated by the donor. The donor will deposit an agreed one-time sum with the Trust, which will be added to the construction fund. In return, the designated older person will receive a licensed, non-transferable right of lifelong residence. This is a guaranteed right to receive accommodation services; it does not transfer ownership of land or a room.

Monthly fees: The one-time payment secures the right of residence only. The Board of Trustees will determine whether monthly fees for food and day-to-day services remain payable and whether any reduction will apply.

Temporary vacancy: If the resident is temporarily absent and the room is rented, the related income will be shared on the proposed basis of 75 per cent to the resident/family and 25 per cent to the Trust.

After death: Following the resident's death, the room will revert to the Trust and will be used to serve another older person in need.

10(b). Sadaqah Jariyah - Memorial Room Sponsorship

A donor may sponsor a designated room in memory of a deceased parent, spouse, or other loved one. The deceased person's name will be inscribed on a plaque at the room entrance. The one-time donation will be added to the construction fund. If this programme generates a permanent fund, the fund's income or returns will subsidise the monthly fees of older residents who lose their financial means after living at the centre for an extended period.

Spiritual significance: Under Islamic Shariah, the merit of Sadaqah Jariyah continues to accrue to a donor after death for as long as the donated asset is used for public benefit.

10(c). General Donations

Donors who do not wish to participate through either of the arrangements above may contribute directly to construction. Donations of any amount are welcome and will be applied to the most urgent item within the construction budget in Section 8. Institutional donors and businesses may also contribute through corporate social responsibility (CSR) programmes.

Policy on the use of funds: In accordance with the Trust deed, surplus income will be used only for social welfare, Sadaqah Jariyah subsidies, and environmental conservation, and never for private purposes. Expenditure will be reviewed at regular meetings of the Board of Trustees and presented to donors when required.

11. Proposed Implementation Schedule

Phase	Activity	Estimated duration
Phase 1	Finalise design, conduct soil testing, and obtain approvals	1-2 months
Phase 2	Construct foundation and ground floor	4-5 months
Phase 3	Construct first floor and cast roof	3-4 months
Phase 4	Complete finishing, electrical and water lines, and solar installation	2-3 months
Phase 5	Install furniture and equipment, recruit staff, and prepare for opening	1-2 months

12. Transparency and Accountability

Mitali Hossain Trust is committed to ensuring the proper use of every donation. Donors will receive regular updates on construction progress; expenditure records will be maintained; and financial use will be reviewed at regular meetings of the Board of Trustees. Separate progress reports will be made available to major donors when required.

13. Appeal for Partnership

Shesher Kobita Ananda Ashram is more than a building. It represents a responsibility to a generation that has the right to live in dignity and security in later life.

I began this journey by donating my own land. What is now required is one-time support to construct the building. Once established, the centre is intended to stand on its own operational footing and provide shelter to isolated and vulnerable older persons who have no one beside them. Your financial support and counsel will strengthen this endeavour.

Respectfully,

Mitali Hossain

Chair, Mitali Hossain Trust

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Bank and branch	Sonali Bank, Dhuptara Branch

Note: This English document is a formal translation and donor-facing reformatting of the Bengali project proposal supplied by Mitali Hossain Trust. Quantitative statements, budget figures, governance information, and visual materials have been retained from that source.